

The Untreated: America's Mental Health Crisis in Data

One Hundred Years — Report 06 of 10 Window: 1900-2024

Abstract

In 1955, the United States maintained 558,922 psychiatric beds in state hospitals — 337 beds for every 100,000 citizens. By 2024, that number had fallen to 37,650, or approximately 11 per 100,000: a 97% per-capita decline. This report documents the consequences of deinstitutionalization — the systematic closure of state psychiatric hospitals without adequate construction of the community-based treatment systems that were promised as replacements. The data reveals a transfer, not a solution: patients moved from hospitals to jails, shelters, emergency rooms, nursing homes, and sidewalks. The cost arithmetic is unambiguous — America spends 19 times more to incarcerate a person with serious mental illness than to treat them in the community.

I. The Beds

The scissors chart tells the story in two lines. One falls; the other rises.

In 1900, the United States housed approximately 150,000 people in state psychiatric hospitals — roughly 200 beds per 100,000 population. The system grew through the first half of the twentieth century as states built large custodial facilities, reaching a peak of 558,922 beds in 1955 (337 per 100,000).

The decline began with the introduction of chlorpromazine (Thorazine) in 1955 and accelerated after President Kennedy signed the Community Mental Health Act in 1963. The act promised a network of 2,000 community mental health centers to replace institutional care. Fewer than half were built. Most were underfunded within a decade.

By 1972, the incarceration rate per 100,000 exceeded the psychiatric bed rate for the first time. The lines have never re-crossed. By 2024, psychiatric beds had fallen to 11 per 100,000 while incarceration stood at 531 per 100,000.

Key finding: The 97% per-capita decline in psychiatric beds is the largest single reduction in healthcare capacity in American history. It occurred through deliberate policy — not through a reduction in need.

II. The Transfer

The 521,272 beds that disappeared between 1955 and 2024 represent people. Where did they go?

Jails and prisons. An estimated 383,000 people with serious mental illness (SMI) are incarcerated at any given time — roughly ten times the number in state psychiatric hospitals. The three largest psychiatric facilities in America are the Los Angeles County Jail, Cook County Jail, and Rikers Island. In state prisons, 37% of inmates have a diagnosed mental health condition. In local jails, the figure is 44%.

Homelessness. Approximately one-third of the 653,000 people experiencing homelessness on any given night have a serious mental illness — roughly 172,000 people. Among the unsheltered population, the rate rises to 39%.

Nursing homes. The Medicaid IMD exclusion — which bars federal payment for psychiatric hospitals with more than 16 beds — created a perverse incentive to shift patients to nursing homes. An estimated 200,000 nursing home residents have SMI as a primary diagnosis, housed in facilities not designed for psychiatric care.

Emergency rooms. Twelve million psychiatric ER visits occur annually. Average boarding time exceeds 8 hours. Repeat visit rates approach 40%. Emergency departments have become de facto psychiatric holding facilities.

Community (unsupported). Approximately 5 million adults with SMI receive no treatment whatsoever. The average delay between symptom onset and first treatment — the “treatment gap” — is 11 years.

III. The Crossover

The crossover year — 1972 — marks when America’s incarceration rate first exceeded its psychiatric bed rate per capita. This is not a coincidence of timing. Multiple researchers have documented the relationship:

- Penrose (1939) first identified the inverse relationship between psychiatric hospital and prison populations.
- Steadman et al. (1984) documented the “hydraulic” relationship between mental health and criminal justice systems.
- Raphael & Stoll (2013) estimated that deinstitutionalization accounted for 4-7% of the increase in incarceration between 1980 and 2000.

The causal mechanism is not direct. Deinstitutionalization did not cause mass incarceration — the War on Drugs, mandatory minimums, and changes in sentencing policy were primary drivers. But the closure of psychiatric hospitals removed a system that had absorbed a significant population of people with SMI. When that system disappeared, the criminal justice system absorbed them by default.

IV. The Cost

The cost comparison is not subtle:

System	Annual Cost per Person
Outpatient mental health	\$4,500
Supported housing	\$18,000
State psychiatric hospital	\$32,000
Nursing home	\$52,000
County/city jail	\$60,000
State prison	\$85,000
ER psychiatric boarding	\$2,200/day (\$803,000 annualized)

America spends approximately \$15 billion annually on criminal justice costs for people with SMI. Federal community mental health spending is approximately \$5.5 billion. The system spends roughly three times more to incarcerate people with mental illness than to treat them.

The arithmetic: For the cost of incarcerating one person with SMI in a state prison (\$85,000/year), the system could provide outpatient treatment to 19 people (\$4,500/year each). This is not a policy argument. It is division.

V. The Encounter

People with untreated serious mental illness are 16 times more likely to be killed during a law enforcement encounter than the general population (Treatment Advocacy Center, 2015).

Approximately 25% of fatal police shootings involve people showing signs of mental illness — roughly 274 people per year over the past decade.

Two million times annually, someone calls 911 for a mental health crisis. Police respond because no one else is available. Roughly 10% of all police calls involve mental health crises, consuming 2-4 hours of officer time per incident at a collective cost exceeding \$900 million per year.

Alternatives exist and are measurable: - Crisis Intervention Teams (CIT) reduce arrests by 58% and use of force by 28%. - Co-responder models (mental health professional + officer) divert 60% of cases from ERs. - The 988 Suicide & Crisis Lifeline handled 5.7 million calls and 2.2 million texts in 2023.

VI. The 11-Year Treatment Gap

The average delay between the onset of mental health symptoms and first treatment is 11 years. For most people, symptoms begin in adolescence — ages 14-16 for anxiety and mood disorders, late teens for psychotic disorders. Treatment, when it arrives, typically comes in the mid-twenties.

During those 11 years, the untreated person cycles through systems that cannot help them: school discipline, juvenile justice, emergency rooms, homelessness, and incarceration. Each contact compounds the original condition. By the time treatment begins — if it begins — the damage has accumulated across a decade of institutional failure.

One in six American children aged 6-17 has a diagnosable mental health condition. Half receive no treatment. Among youth in juvenile detention, 70% have a diagnosable mental health condition. The school-to-prison pipeline is, in significant part, a treatment-gap pipeline.

VII. Limitations

This report is subject to several important constraints:

1. **Causal complexity.** The relationship between deinstitutionalization and incarceration is correlational, not simply causal. Multiple factors contributed to mass incarceration independently of hospital closures.
2. **Definition drift.** The definition of “serious mental illness” has changed over time. Historical comparisons necessarily involve imprecision.
3. **Counting problems.** Homelessness counts (PIT methodology) are known to undercount. Jail mental health statistics depend on screening quality that varies by facility.
4. **Cost variability.** Per-person costs vary significantly by state, facility type, and patient acuity. National averages obscure wide ranges.
5. **The 16x statistic.** The Treatment Advocacy Center’s 2015 finding has been questioned on methodological grounds, though the directional finding — dramatically elevated risk — is supported by subsequent research.

VIII. Conclusion

America did not choose between treating mental illness and ignoring it. It chose to stop treating mental illness in hospitals and then failed to treat it anywhere else. The patients did not disappear. They were transferred — to jails at \$85,000 per year, to emergency rooms at \$2,200 per day, to sidewalks at incalculable human cost.

The data does not argue for any particular solution. It documents a specific failure: the largest reduction in healthcare capacity in American history, executed without building the replacement systems that were promised. Sixty-nine years after the peak, the consequences are measured in 383,000 incarcerated people with SMI, 172,000 homeless people with SMI, 12 million psychiatric ER visits per year, and an 11-year average gap between illness and treatment.

The beds went away. The illness did not.

Data sources: SAMHSA, BJS National Prisoner Statistics, Treatment Advocacy Center, HUD AHAR, Washington Post Fatal Force Database, CDC, NAMI, DOJ Bureau of Justice Statistics, HRSA, CMS, Vera Institute of Justice.

Confidence system: This report uses a three-tier confidence system (High / Candidate / Speculative). Each claim is tagged in the interactive visualization. The methodology tab provides full transparency on data provenance and known limitations.

Any errors of methodology, interpretation, or framing are the author's alone.

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